Reg. No. RMEE2552994
PURE PATH LAB
Add: Infront of Cantabil, Beside Distt. Hospital-Gonda
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PURE PATH LAB
Palce of Accuracy & Your Satisfaction
Dr. Abha Mishra
M.B.B.S (M.D. PATH)
Reg. No-MP-8779

**Mr. Nitish Kashyap**
Age / Sex : 25 YRS / M
Reg. no. : 6979

Registered on : -
Collected on : -
Received on : -
Reported on : -

## HAEMATOLOGY

| TEST | | VALUE | UNIT | REFERENCE |
|---|---|---|---|---|
| **HEMOGLOBIN** | **H** | **18.1** | **g/dl** | **13 - 17** |
| TOTAL LEUKOCYTE COUNT | | 9,400 | cumm | 4,800 - 10,800 |
| NEUTROPHILS | | 67 | % | 40 - 80 |
| LYMPHOCYTE | | 23 | % | 20 - 40 |
| **EOSINOPHILS** | **H** | **08** | **%** | **1-6** |
| MONOCYTES | | 02 | % | 2 - 10 |
| BASOPHILS | | 00 | % | <2 |
| **PLATELET COUNT** | **L** | **1.00** | **lakhs/cumm** | **1.5 - 4.1** |
| TOTAL RBC COUNT | | 4.91 | million/cumm | 4.5 - 5.5 |
| **HEMATOCRIT VALUE, HCT** | **H** | **50.1** | **%** | **40 - 50** |
| **MEAN CORPUSCULAR VOLUME, MCV** | **H** | **102.0** | **fL** | **83 - 101** |
| **MEAN CELL HAEMOGLOBIN, MCH** | **H** | **36.9** | **Pg** | **27 - 32** |
| **MEAN CELL HAEMOGLOBIN CON, MCHC** | **H** | **36.1** | **g/dl** | **31.5 - 34.5** |
| MEAN PLATELET VOLUME, MPV | | 10.8 | fL | 6.5 - 12 |
| **R.D.W. - SD** | **H** | **53.6** | **fL** | **39 - 46** |
| **R.D.W. - CV** | **H** | **15.3** | **%** | **11.6 - 14** |
| P-LCR | | 33.7 | 19.7 | - 42.4 |
| **P.D.W.** | **H** | **16.9** | **fL** | **9.6 - 15.2** |

**Possible causes of abnormal parameters**

| | High | Low |
|---|---|---|
| **RBC, Hb, or HCT** | Dehydration, polycythemia, shock, chronic hypoxia | Anemia, thalassemia, and other hemoglobinopathies |
| **MCV** | Macrocytic anemia, liver disease | Microcytic anemia |
| **WBC** | Acute stress, infection, malignancies | Sepsis, marrow hypoplasia |
| **Platelets** | Risk of thrombosis | Risk of bleeding |

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## BIOCHEMISTRY

| TEST | VALUE | UNIT | REFERENCE |
|---|---|---|---|
| SERUM UREA | 38.57 | mg/dl | 19 - 45 |
| SERUM CREATININE | 1.03 | mg/dl | 0.72 - 1.43 |
| SERUM PHOSPHORUS | 4.45 | mg/dl | 2.5 - 4.5 |
| SERUM URIC ACID | 6.87 | mg/dl | 3.5 - 7.2 |
| SERUM SODIUM | 136.1 | mmol/L | 136 - 146 |
| SERUM POTASSIUM | 4.55 | mmol/L | 3.5 - 5.1 |
| ICALCIUM | 1.15 | mmol/l | 1.13 - 1.33 |

Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

| Causes of Increased Serum Creatinine Level | Causes of Decreased Serum Creatinine Level |
|---|---|
| 1. Pre-renal, renal, and post-renal azotemia | 1. Pregnancy |
| 2. Large amount of dietary meat | 2. Increasing age (reduction in muscle mass) |
| 3. Active acromegaly and gigantism | |

| | | | | |
|---|---|---|---|---|
| **SERUM BILIRUBIN (TOTAL)** | **H** | **1.34** | **mg/dl** | **0.2 - 1.2** |
| **SERUM BILIRUBIN (DIRECT)** | **H** | **0.31** | **mg/dl** | **0 - 0.3** |
| **SERUM BILIRUBIN (INDIRECT)** | **H** | **1.03** | **mg/dl** | **0.2 - 1** |
| **SGOT (AST)** | **H** | **75.14** | **U/l** | **0 - 37** |
| **SGPT (ALT)** | **H** | **80.67** | **U/l** | **13 - 40** |
| SERUM ALKALINE PHOSPHATASE | | 98.47 | U/l | 44 - 147 |
| SERUM PROTEIN | | 7.14 | g/dl | 6.4 - 8.3 |
| **SERUM ALBUMIN** | **H** | **5.38** | **g/dl** | **3.5 - 5.2** |
| **GLOBULIN** | **L** | **1.76** | **g/dl** | **1.8 - 3.6** |
| **A/G RATIO** | **H** | **3.06** | **1.1** | **- 2.1** |

**LFT Interpretation**
Liver Function Blood Test gives an insight into your liver health and helps identify problems like hepatitis, cirrhosis, and fatty liver disease, which may cause similar symptoms but require different treatments to recover.

**Test Significance**
Besides diagnosing liver problems, LFT's also monitor overall liver functioning. Monitoring helps people with liver disease or taking medication, as it helps screen whether the treatment works fine or requires adjustments. Moreover, Liver Function Tests help determine if someone is at risk of developing liver diseases. Apart from assessing your chances, this test also checks the severity of the liver damage to help the doctor plan and prescribe appropriate treatment.
**Increased in:** Acute or chronic hepatitis, cirrhosis, biliary tract obstruction, toxic hepatitis, neonatal jaundice (neonatal hyperbilirubinemia), congenital liver enzyme abnormalities (Dubin-Johnson, Rotor, Gilbert, Crigler-Najjar syndromes), fasting, hemolytic disorders. Hepatotoxic drugs.

| | | | |
|---|---|---|---|
| RANDOM BLOOD SUGAR | 104.5 | mg/dl | 70 - 140 |

~~~ End of report ~~~
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